



Policy

Reducing Restrictive Practices in Mental Health Services.

Policy code EMHS: 325

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Version 1

Functional subgroup Mental Health

Summary

East Metropolitan Health Service (EMHS) is committed to reducing, and, where possible, eliminating the use of the restrictive practices of seclusion and restraint in all inpatient mental health services. EMHS mental health services must develop an action plan to reduce, and where possible eliminate, use of restrictive practices, consistent with this policy and the Six Core Strategies for Reducing Seclusion and Restraint.

Policy owner Executive Director Mental Health

Contact

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NSQHS Standards (second edition) Standard 1.16

Impact Statement Declaration (ISD) The voice of the Aboriginal community is reflected in this policy through Aboriginal consultation to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record ID 3781).

Status Active

1. Background

1.1. About this document

East Metropolitan Health Service (EMHS) is committed to reducing, and, where possible, eliminating the use of the restrictive practices of seclusion and restraint in all inpatient mental health services, as per the [Chief Psychiatrist's Standards for Clinical Care](#).

EMHS mental health services must develop, implement, and annually review a service level action plan to prevent, reduce and, where safe and possible, eliminate the use of seclusion and restraint, in collaboration with staff, lived experience representatives, carers and support people. Plans must be consistent with the principles and requirements identified in this policy, including compliance and monitoring requirements.

1.2. Policy applicability

1.2.1. Authorised mental health areas.

This policy applies to the use of the restrictive practices of seclusion and restraint, as authorised under the Mental Health Act 2014 (MHA 2014), within authorised mental health services.

1.2.2. Non-authorised mental health areas

This policy applies to non-authorised mental health services, which are governed by EMHS mental health services (e.g., Mental Health Emergency Centres, Ward 2K, Bidi Wungen Kaat Centre). These services must develop and implement action plans to reduce their use of restrictive practices as per the information outlined in this policy.

Use of restrictive practices in non-authorised mental health services which are governed by EMHS mental health services is subject to common law, the [WA Department of Health Use of Restrictive Practices in Non-Authorised Health Care Settings Policy](#), and the MHA 2014 where applicable.

1.2.3. Non-authorised non-mental health areas

This policy is **not applicable** to non-mental health areas (e.g., Emergency Departments and general hospital wards). The use of restrictive practices in non-authorised areas is subject to the [WA Department of Health Use of Restrictive Practices in Non-Authorised Health Care Settings Policy](#), and the MHA 2014 where applicable.

Key features of the Use of Restrictive Practices in Non-Authorised Health Care Settings Policy can be found in the [Restrictive Practices Factsheet](#).

1.3. Key definitions

Term	Definition
Restrictive Practices	Interventions that may infringe a person's human rights and freedom of movement, including observation, seclusion, manual restraint, mechanical restraint, and rapid tranquillisation.
Seclusion	As per the WA Mental Health Act 2014, seclusion is the confinement of a person who is being provided with treatment or care at an authorised hospital by leaving the person at any time of the day or night alone in a room or area from which it is not within the person's control to leave.

Physical Restraint	As per the WA Mental Health Act 2014, physical restraint is the restraint of a person by the application of bodily force to the person's body to restrict the person's movement.
Chemical Restraint / Rapid Tranquilisation	Chemical restraint means the giving of a drug to a person for the primary purpose of controlling the person's behaviour by restricting their freedom of movement. Chemical restraint does not include the giving of a drug to a person for the purpose of treatment or medical treatment.
Mechanical Restraint	As per the WA Mental Health Act 2014, mechanical restraint is the restraint of a person by the application of a device (for example, a belt, harness, manacle, sheet, or strap) to a person's body to restrict the person's movement. Mechanical restraint does not include either the appropriate use of medical or surgical appliances or the appropriate use of furniture to restrict a person such as cot sides or a chair fitted with a table. It also does not include physical or mechanical restraint by a police officer.
Restorative just and learning culture	A restorative just and learning culture focuses on harm done rather than blame. The approach recognises that people make mistakes, while ensuring people are held accountable for their decisions. This approach aims to repair trust and relationships damaged after an incident. A restorative just and learning culture seeks to heal, learn, and improve by asking: • What happened? • Who has been harmed? • What are their needs? • Whose obligation is it to meet those needs? • How can things be put right?
System failure	System failure focuses on what went wrong in the system as a whole to result in an adverse outcome. Investigating system failure seeks to identify gaps in the healthcare system and processes that may have contributed to the occurrence of an event.
Treatment failure	Treatment failure refers to the non-therapeutic and potentially harm causing nature of restrictive practices. Treatment failure acknowledges that restrictive practices do not support people towards recovery and do not help people to better manage the thoughts and emotions that can trigger behaviours that may injure themselves or others. Rather, restrictive practices are safety measures of absolute last resort.

2. EMHS Vision for Restrictive Practices in mental health

The use of seclusion and restraint potentially creates significant risks for people accessing mental health services, their carers and support people, and mental health service staff. These risks may include physical harm, psychological trauma, loss of dignity, cultural harm, breakdown in therapeutic relationships and injury.

The following principles underpin EMHS' commitment to reducing, and where possible eliminating, the use of seclusion and restraint in a way that supports good clinical practice, optimal care provision and provides a safe environment for all concerned:

- Prioritisation of human rights supported decision making, and trauma informed care principles.
- Safety for people receiving care, carers and support people, and staff.

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- Full and formal inclusion of the lived experience voice
- Acknowledgement of restrictive practices as **a system failure/treatment failure**. This acknowledgement emphasises the **system responsibility** to provide an environment and treatments which are conducive to reduction/elimination of restrictive practices and provide a safe environment for people receiving care, carers and support people, and staff.
- A restorative, just, and learning culture, where incidents of seclusion and restraint are viewed as learning opportunities to improve service delivery.

3. Restrictive Practices Reduction Action Plans

The EMHS commitment to reducing, and where possible eliminating, the use of seclusion and restraint will be operationalised through the development and implementation of service level action plans. Action plans must align with the EMHS vision and principles outlined in this policy and address the Six Core Strategies for Reducing Seclusion and Restraint.

Action Plans must be developed for each EMHS mental health inpatient service using the Restrictive Practices Reduction Action Plan template.

3.1. The Six Core Strategies for Reducing Seclusion and Restraint

The Six Core Strategies for Reducing Seclusion and Restraint Use (The Six Core Strategies) were developed by the USA National Association of Mental Health Program Directors (NASMHPD) through extensive literature reviews and expertise from those who have successfully reduced the use of restrictive practices in mental health settings. Figure 1 shows an EMHS adapted version of the Six Core Strategies.

The Six Core Strategies provides services with an evidence-based systems approach to support the provision of least restrictive practice. The Six Core Strategies are a trauma informed, recovery based, collaborative approach to reducing seclusion and restraint, helping to improve the experiences and outcomes of people accessing mental health services, their carers and support people, and staff.

Figure 1. The Six Core Strategies for Reducing Seclusion and Restraint Use



Leadership toward organisational change

- Leadership must be aligned and committed to supporting, applying, and resourcing seclusion and restraint reduction initiatives.
- Consistent and continuous involvement of leadership at all levels

Full inclusion of lived experience

- Inclusion of lived experience in event oversight, monitoring, debriefing, and committees.
- Integration of lived experience in various roles and levels in mental health services, with Executive support and advocacy.

Using data to inform practice.

- Data is to be collected at a ward/unit level to identify trends (e.g., shift, day, time, staff member involvement, person factors).
- Sites must actively review and critically reflect on seclusion and restraint data and utilise data to inform reduction activities.

Workforce development

- Supporting staff to create a treatment environment where policy, procedures, and practices are recovery focused, trauma informed, and less likely to trigger conflict or require the use of restrictive practices.
- Ensuring staff are equipped with the required knowledge, skills, and abilities to support reduction of restrictive practices.

Use of seclusion and restraint reduction tools

- The integration of tools and assessments into facility policy and procedures and each individual person's Treatment, Support and Discharge Plan.
- May also include environmental changes such as implementation of comfort rooms and sensory rooms, or any other meaningful clinical interventions which support people in emotional self-management.

Post incident support and learning

- Post incident support focuses on the immediate physical and emotional wellbeing of people involved in the incident.
- Post incident learning helps gain information to understand what happened and why, and to learn about how to improve care and support provided to the person involved.
- Together, post incident support and learning make up post incident debriefing. It is imperative that senior clinical and medical staff participate in the debriefing process.

The Six Core Strategies Fact Sheets and the EMHS Six Core Strategies Service Review Tool provide further information to support services in developing their Restrictive Practices Reduction Action Plan and implementing The Six Core Strategies.

3.2. Cohorts for Special Consideration

Seclusion and Restraint Reduction Action Plans must consider the needs of different groups of people, including:

- Youth
- Older adults
- People who are prisoners of WA Police Force or Department of Correctional Services where statutory requirements exist, and obligation to public safety and maintaining custody override medical need.
- LGBTIQI+SB people
- Aboriginal and Torres Strait Islander people
- People with ethnoculturally and linguistically diverse backgrounds
- People with disability, including cognitive and intellectual disability and neurodiverse people.

Consideration of needs includes, but is not limited to:

- Gender
- Culture
- Spirituality
- Trauma
- Communication

4. Documentation

Restrictive Practices Reduction Action Plans must be documented on the Restrictive Practices Reduction Action Plan Template.

5. Compliance monitoring and legislative penalties

5.1. Compliance monitoring

Each EMHS Site/Service/Group Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

Each EMHS mental health Service Director or Co-Directors are to ensure compliance with this policy and be accountable for the development and implementation of Restrictive

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Practices Reduction Action Plans.

- Compliance is to be monitored via the EMHS Mental Health Leadership Committee. Action Plans are to be tabled and progress report provided at the EMHS Mental Health Leadership Committee quarterly.
- Site based seclusion and restraint trends are to be tabled at every EMHS Mental Health Leadership Committee for monitoring, including rates and duration of seclusion and restraint events.
- Each EMHS mental health Service Director or Co-Directors are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

5.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

6. Legislation

[Mental Health Act WA 2014](#)

7. Additional reference

7.1. WA Health policies

- [Safety Planning for Mental Health Consumers Policy](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People who may be Suicidal](#)
- [Principles for the Care of People who may be at Risk of Violent or Aggressive Behaviour](#)
- [Clinical Incident Management Policy](#)
- [Statewide Standardised Clinical Documentation for Mental Health Services](#)

7.2. EMHS policies

- [Emergency Psychiatric Treatment Policy](#)
- [Mental Health Advocates Policy](#)
- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [Rights of Carers and Other Personal Support Persons Policy](#)
- [Rights of Patients in Mental Health Policy](#)
- [Sexual Safety in Mental Health Policy](#)

7.4 Others

- [Six Core Strategies for Reducing Seclusion and Restraint Use - A Snapshot of Six Core Strategies for the Reduction of S/R. National Association of State Mental Health Program Directors](#)
- [Australian Institute for Health and Welfare – Seclusion and restraint in mental health care](#)
- [National Mental Health Commission – Reducing Restrictive Practices](#)
- [National Principles for Communicating about Restrictive Practices with Consumers and Carers](#)
- [Least Restrictive Practice – Te Pou](#) – a suite of resources and information related to least restrictive practice developed the by national workforce centre for mental health, addiction, and disability in New Zealand.
- [Chief Psychiatrist's Guideline issues under Sections 547 \(3\) and 548 of the Mental Health Act 2014: Use of Detention and Restraint in Non-Authorised Healthcare Settings](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)